



Current Issues in Ethics in the Integration of AI in Healthcare

Jasper Kyle Catapang, MA

Doctoral Candidate

Graduate School of Global Studies

Tokyo University of Foreign Studies

AI Engineer Lead

AI Product Development Division

Money Forward Japan

About me

Research areas

- Natural language processing (NLP), AI safety, AI governance, computational linguistics

Education

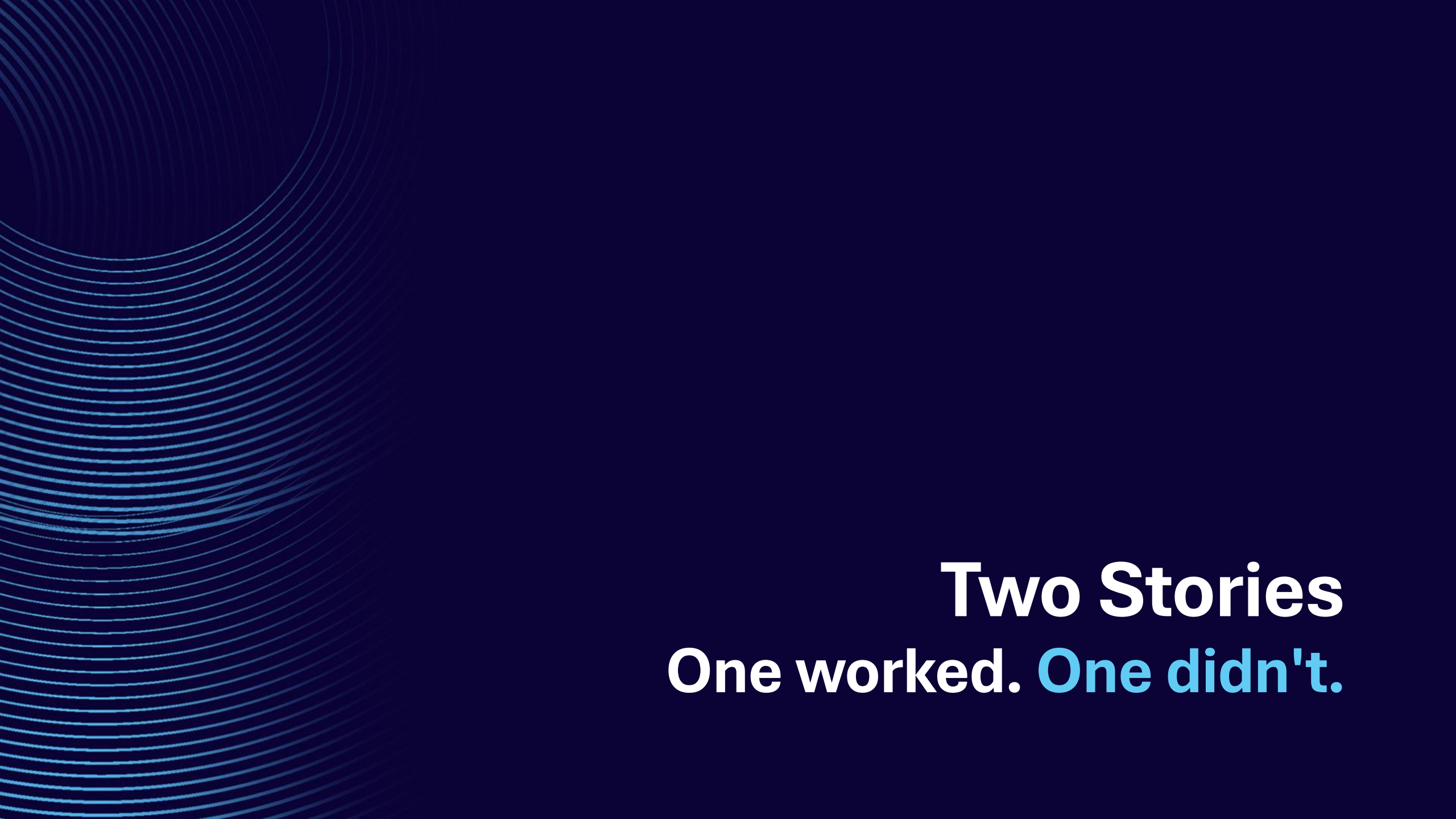
- Tokyo University of Foreign Studies – PhD in Global Studies (ongoing)
- University of Birmingham – MA Applied Linguistics
- UP Manila – BS Computer Science (Health Informatics)

Select research papers

1. Catapang, J.K., (2026). Building the Ethical AI Framework of the Future: From Philosophy to Practice, AI and Ethics.
2. Catapang, J.K., (2025). Explaining Bias in Internal Representations of Large Language Models via Concept Activation Vectors, NLDB.
3. Isip-Tan, ... , Catapang, J.K., (2023). Interdisciplinary Approach to Identify and Characterize COVID-19 Misinformation on Twitter: Mixed Methods Study, JMIR Formative Research.
4. Catapang, J.K., et al., (2023). Improving detection of diabetic retinopathy in low-resolution images via latent diffusion, MIPR.

Overview

1. Two Stories
2. The Map
3. What Breaks
4. Who's Watching
5. Not a Footnote
6. The Checklist



Two Stories
One worked. One didn't.

It Worked: TB Screening, Manila

- WHO-recommended AI-based Computer-Aided Detection (AI-CAD) since 2021
- Médecins Sans Frontières (MSF), Tondo, Manila—since Oct 2022
- 20,000+ screened; ~5% confirmed Tuberculosis (TB)



It Didn't: Epic Sepsis Model

- Widely implemented, US hospitals
- Area Under the Curve (AUC) 0.63
- Sensitivity 33%
- Missed most true sepsis cases (high false negatives)



The Algorithm **Wasn't** the Difference

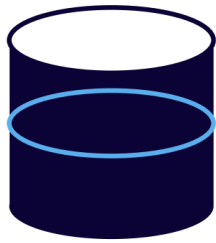
It was whether anyone checked—here—
before it reached patients.



The Map

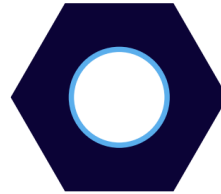
Three kinds of **AI** research

Which Kind of **AI Research**?



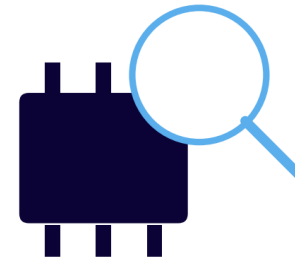
With Data

Point ML at
a dataset



With AI Tools

AI does the
research



On AI Tools

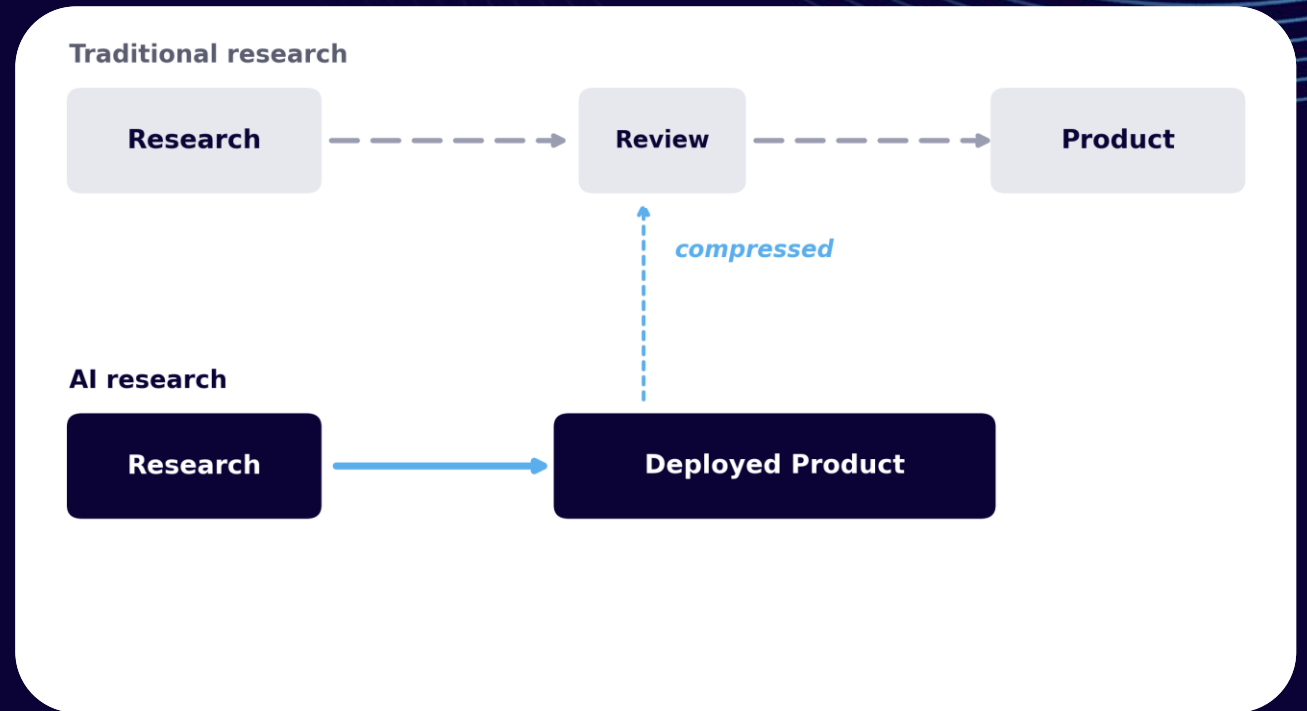
AI is what's
studied

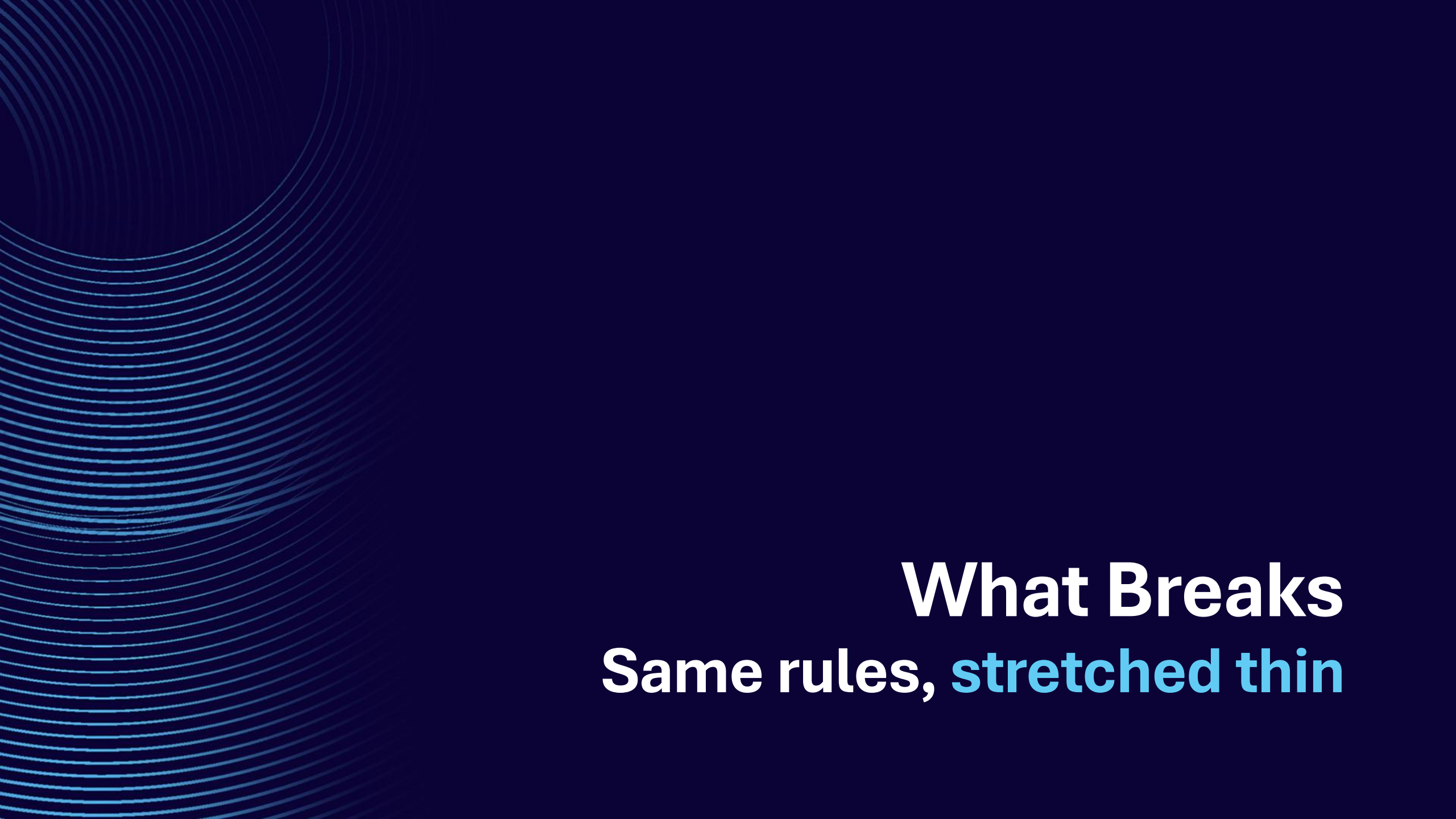
Which One Are You?

- ☐ With data
- ☐ With Artificial Intelligence (AI) tools
- ☐ On AI tools
- ☐ More than one

Why AI Research Is Different

- Research reaches product fast
- Low barrier to entry
- Risk not caught reaches patients





What Breaks
Same rules, **stretched thin**

Consent

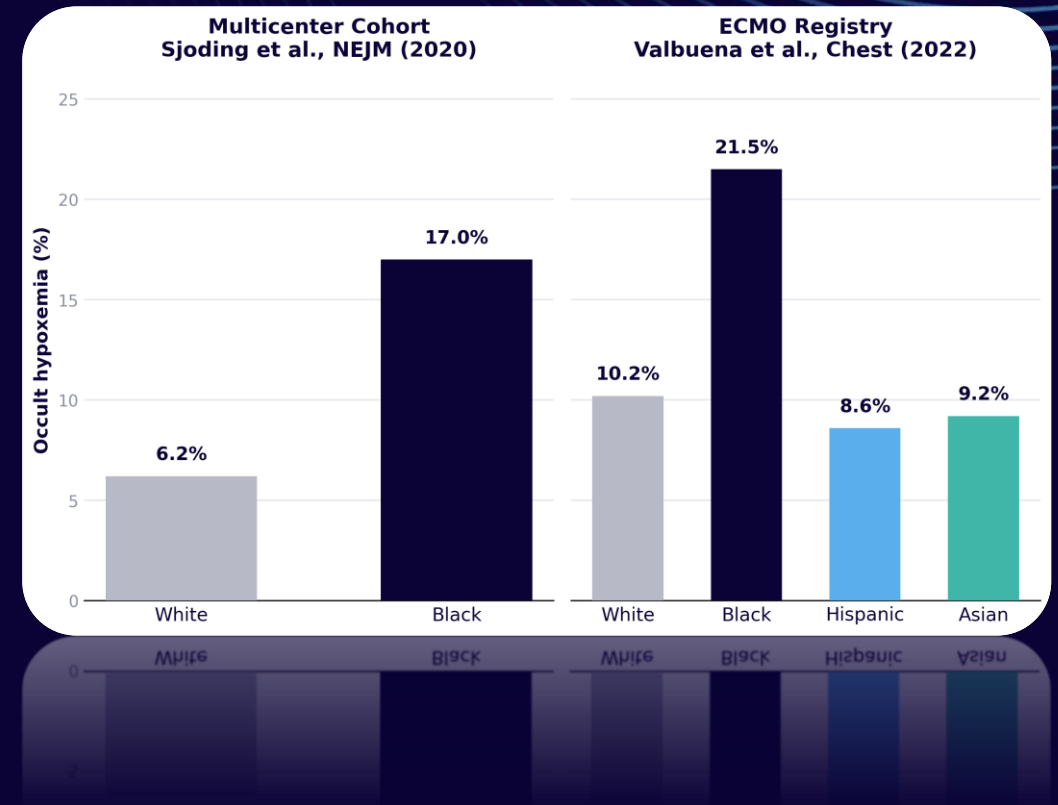
- Collected for care, reused for training
- National Ethical Guidelines for Research Involving Human Participants (NEGRIHP) already covers future use
- Would they recognize this use?

Privacy

- Republic Act (RA) 10173 and the National Privacy Commission (NPC) still apply
- Privacy impact assessment required
- Breach notice: 72 hours

Validity & Bias

- Epic: trained well, failed elsewhere
- Datasets often exclude Low- and Middle-Income Country (LMIC) populations
- Pulse ox: biased for Black patients—not Asian



Did You **Declare It?**

- Drafting protocols, screening literature
- Coding Focus Group Discussion (FGD) transcripts
- WHO: disclose tools, prompts, performance

Who Watches Year Three?

- Food and Drug Administration (FDA) draft circular: registers, doesn't monitor
- Model drift isn't anyone's job yet

Five Questions

1. Consent—still covers this?
2. Privacy—where do the data go?
3. Validity—tested on Filipinos?
4. Disclosure—declared your AI use?
5. Accountability—who watches after?



Who's Watching RECs, and beyond RECs

Why RECs **Struggle** With AI

- Review happens too early
- Multi-site coordination is weak
- Private research may skip review entirely

The Exemption Gap

- NEGRIHP exempts “publicly available data”
- WHO: even anonymized data carries AI risk
- Bias and hallucination aren’t on the intake form

Beyond RECs

- Funders, journals, regulators, data hubs
- Philippines (PH): National Artificial Intelligence Center for Research and Innovation (NAICRI) launched Feb 2026

Already Moving

- NEGRIHP 2022 already covers AI research
- AI Development & Regulation Act—committee stage

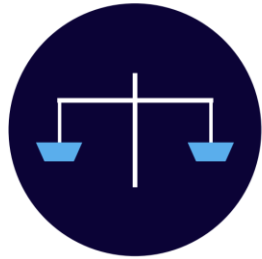


Photo from the Office of Sen. Pia Cayetano

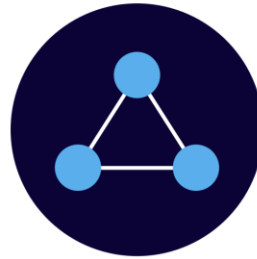


Not a Footnote
The Philippines in this picture

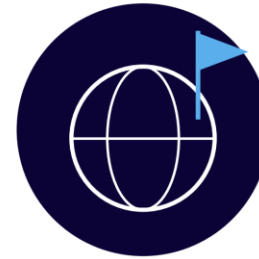
Six LMIC Concerns



Fairness



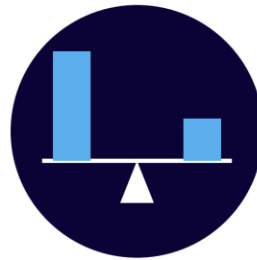
**Benefit
Sharing**



**Data
Colonialism**



**Ethics
Dumping**



**Power
Inequity**



**Capacity-
Building**

Will It **Come Back** Here?

- Built elsewhere may never be optimized for us
- WHO's own Research Ethics Committee (REC) restricts external use of local data

We Already Have the Authority

- Helsinki Art. 23: REC approval, both countries
- NEGRIHP: PH RECs must review foreign protocols too

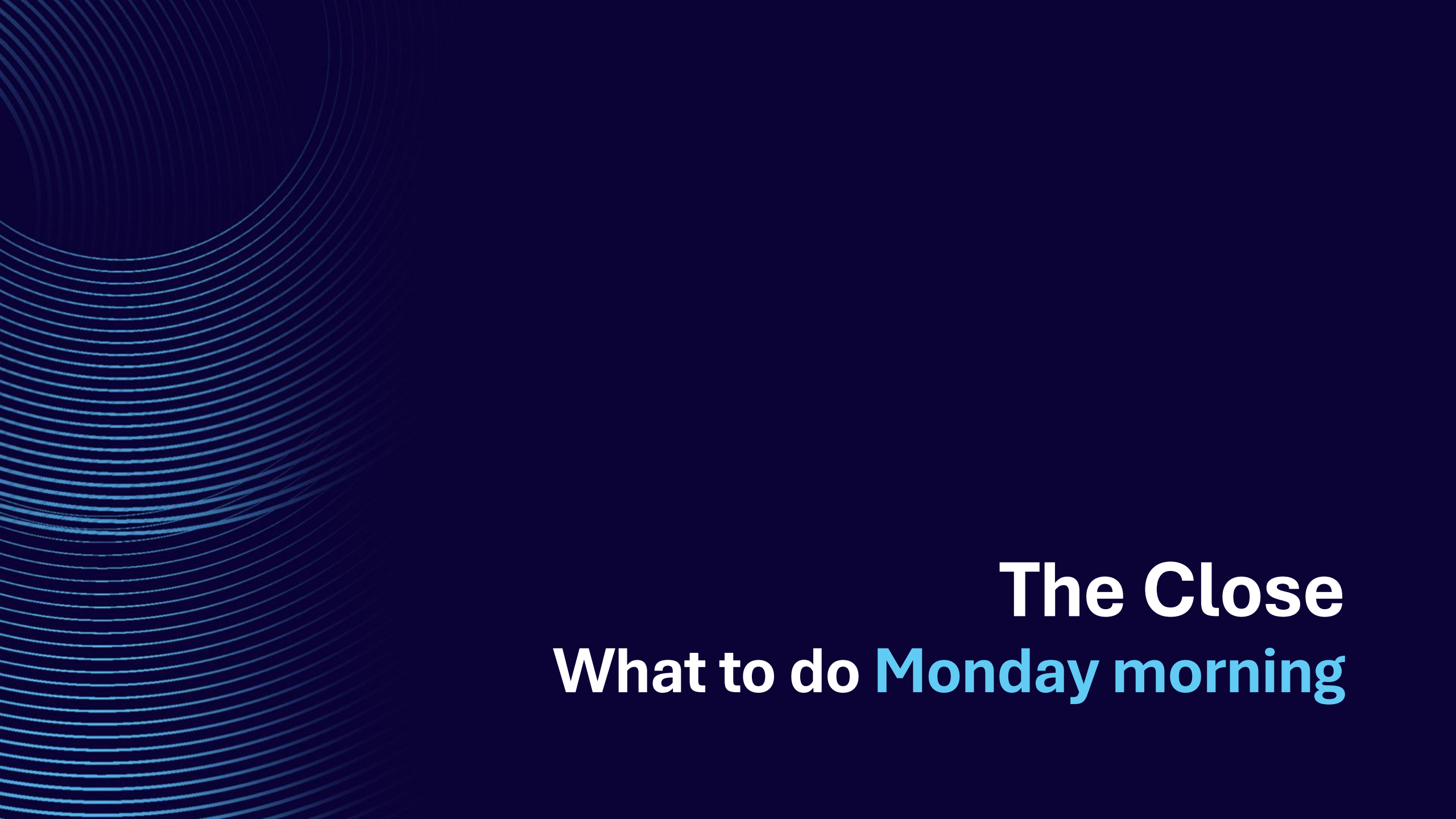
The One Nobody Talks About

- 10,000+ Filipino data annotators
- Fairwork score: 1 / 10
- Crowd worker treatment is a researcher duty



Filipinos Are Doing This Work

- MSF Tondo · PH-led AI-CAD studies
- Department of Science and Technology (DOST) NAICRI · npj Digital Medicine (2025)



The Close

What to do Monday morning

The Checklist

1. Which category is this study?
 2. What were the data originally for?
 3. What population validated this?
 4. Where do the data go?
 5. Which AI tools were used—
declared?
 6. Will it be affordable here?
1. Who labelled the data—how?
 2. Who monitors after deployment?
 3. What happens when it fails?
 4. REC review in both countries?

Three Asks

- **Researchers:** declare your AI use
- **RECs:** interrogate “publicly available”
- **Metro Manila Health Research and Development Consortium (MMHRDC):**
build capacity, join the bill

Questions

Jasper Kyle Catapang, MA

Doctoral Candidate, Tokyo University of Foreign Studies

AI Engineering Lead, Money Forward Japan

jasperkylecatapang@gmail.com

LinkedIn: jcatapang

- AI disclosure: Google Search AI was used to assist in the search for sources. Verification, argumentation, and analysis are the author's own. AI was not used to write nor proofread any portion of this presentation.
All em dashes are the author's 🤪
- Image credits: photos by cottonbro studio & RDNE Stock Project (Pexels); charts by the author; background adapted and edited by the author from Vecteezy.

References

1. World Health Organization. Artificial intelligence-related health research: ethics review and oversight. Geneva: WHO; 2026.
2. World Health Organization. WHO consolidated guidelines on tuberculosis. Module 2: screening—systematic screening for tuberculosis disease. Geneva: WHO; 2021.
3. Min J, Halton J, Villegas C, Chua A, Hewison C, Deborggraeve S. Scanned: the global investments in computer-aided detection and ultraportable X-ray for tuberculosis. PLOS Glob Public Health. 2025;5(3):e0004232.
4. Wong A, Otles E, Donnelly JP, Krumm A, McCullough J, DeTroyer-Cooley O, et al. External validation of a widely implemented proprietary sepsis prediction model in hospitalized patients. JAMA Intern Med. 2021;181(8):1065–70.
5. Habib AR, Lin AL, Grant RW. The Epic Sepsis Model falls short—the importance of external validation. JAMA Intern Med. 2021;181(8):1040–1.
6. Philippine Health Research Ethics Board. National Ethical Guidelines for Research Involving Human Participants (NEGRIHP) 2022. Taguig: DOST-PCHRD; 2022.
7. Sjoding MW, Dickson RP, Iwashyna TJ, Gay SE, Valley TS. Racial bias in pulse oximetry measurement. N Engl J Med. 2020;383(25):2477–8.
8. Valbuena VSM, Barbaro RP, Claar D, Valley TS, Dickson RP, Gay SE, et al. Racial bias in pulse oximetry measurement among patients about to undergo extracorporeal membrane oxygenation in 2019–2020: a retrospective cohort study. Chest. 2022;161(4):971–8.
9. Food and Drug Administration (Philippines). Draft guidelines on the regulation of medical device software (MDSW). Muntinlupa: FDA; 2025.
10. DOST-ASTI. Launch of the National Artificial Intelligence Center for Research and Innovation. Quezon City: DOST-ASTI; 2026.
11. Proposed AI bill gains momentum in Congress. Philstar.com. 2026 Jul 14.
12. Philippine BPO policy tracker: AI laws and rules for 2026. BPOAI.ai; 2026.
13. Tan R, Cabato R. Scale AI’s Remotasks workers in the Philippines cry foul over low pay. Washington Post. 2023 Aug 28.
14. Business & Human Rights Resource Centre. Philippines: Scale AI creating ‘race to the bottom’ as outsourced workers face ‘digital sweatshop’ conditions. 2023.
15. Scale cuts off subsidiary’s remote workers in several countries. Computerworld. 2024 Mar.
16. Marquez N, Carpio EJ, Santiago MR, Calderon J, Orillaza-Chi R, Salanap SS, et al. Performance of chest X-ray with computer-aided detection powered by deep learning-based artificial intelligence for tuberculosis presumptive identification during case finding in the Philippines. BMC Glob Public Health. 2025;3:74.
17. Flores GP, Tamayo RLJ, Leong RNF, Biglaen CSM, Uy KNT, Maglente RRO, et al. Diagnostic performance of a computer-aided system for tuberculosis screening in two Philippine cities. Acta Med Philipp. 2025;59(2).
18. Sarmiento RFR, Overgaard SM, Gai C, Overgaard JD, Ohde JW. Guiding responsible AI in healthcare in the Philippines. NPJ Digit Med. 2025;8(1):338.